

CLINICAL TRIAL SUMMARY

Treating early RA: Combination therapy (Drug A + methotrexate)

Study design

1,032 patients with early rheumatoid arthritis (RA) were randomized (1:1) to:

Arm 1

Drug A +
methotrexate

Arm 2

Placebo +
methotrexate

Dosage

Drug A (40 mg EOW)

Methotrexate (MTX) (20 mg weekly)

KEY PATIENT-REPORTED OUTCOMES AT 24 WEEKS

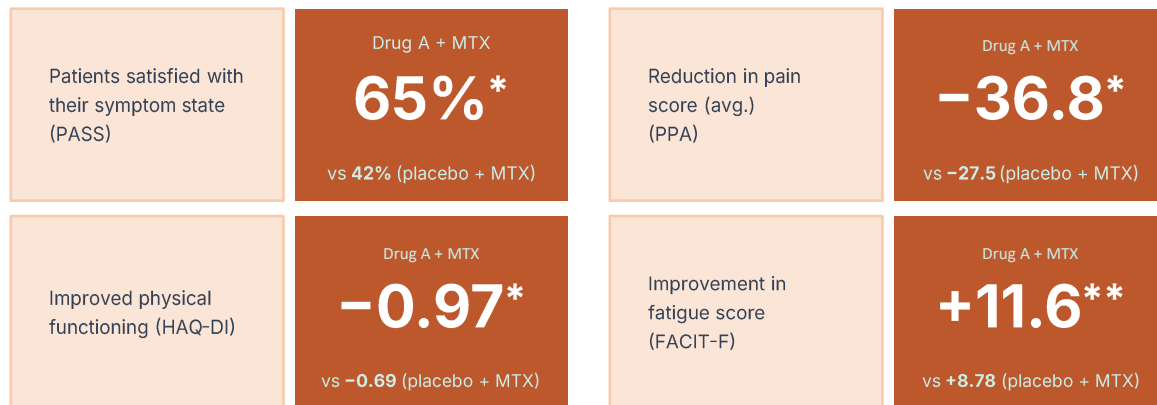
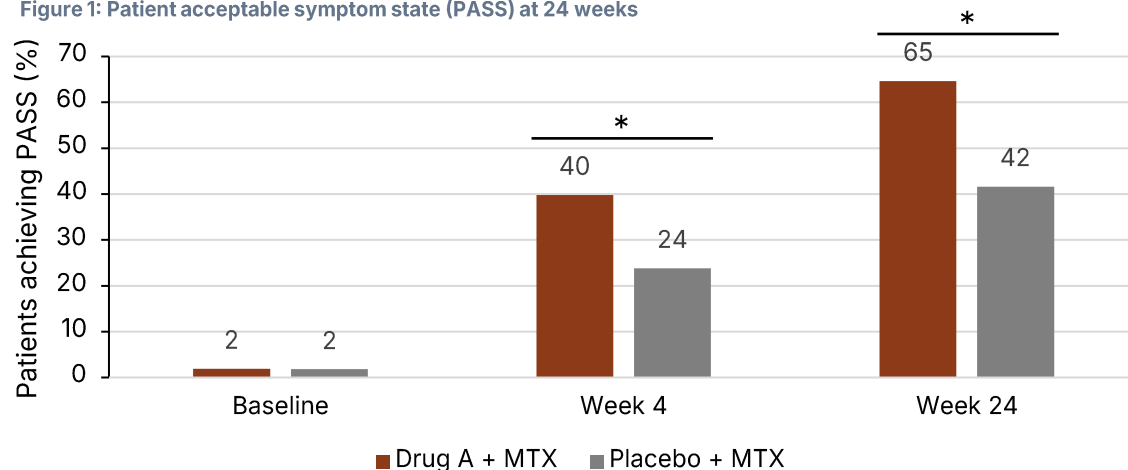


Figure 1: Patient acceptable symptom state (PASS) at 24 weeks



What this means for your patients

Drug A + methotrexate vs placebo + methotrexate (after 24 weeks)

Faster symptom relief

40%*

felt their symptoms were acceptable
after only 4 weeks on combination therapy
(vs. 24% on methotrexate alone)

- Significant pain relief even after 4 weeks
- Patient satisfaction nearly doubles by week 24 (65% vs 42%)*
- Patients feel better and more importantly, **sooner**

Less fatigue, better function

+11.6**

point improvement in fatigue score
vs. +8.8 on methotrexate alone

- Reduced fatigue even from week 4
- Meaningful improvement in physical functioning (HAQ-DI: -0.97 vs -0.69)*
- Patients can more quickly return to doing what they love

Protecting work & daily life

75%

of patients who lost employment were
on methotrexate alone, not combination
therapy (25%)

- Productivity at work is less impaired
- Reduced risk of job loss
- Return to their usual quality of life

For prescribers:

- Benefits are highest in the earlier disease stage (< 6 months), as is the optimal window for prescribing the combination therapy
- Consider prescribing the combination therapy if patients on methotrexate alone remain symptomatic after 4 weeks
- Benefits are practical and patient-reported, use PASS and fatigue as practical check-in questions in your consultation